INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Gemcitabine and cisplatin induction before chemoradiotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00903
Version 1

> Day 1: both drugs; Day 8: gemcitabine. Every 21 days, up to 3 cycles.

Treatment stages

01 **Induction chemotherapy**
Day 1: both drugs; Day 8: gemcitabine. Every 21 days, up to 3 cycles.

02 **Next: chemoradiotherapy**
Separately prescribed

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

> Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Head and neck: nasopharyngeal cancer

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Fever, shivering, sudden illness, unusual bleeding, new breathlessness, persistent vomiting or inability to drink needs urgent oncology advice. Severe chest pain, severe breathing difficulty, collapse or a seizure is an emergency: call 112/999.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

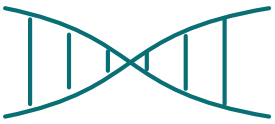
What each medicine does

This chemotherapy is given before chemoradiotherapy to improve cancer control. Induction is the first stage of the planned pathway, and does not replace the later treatment.

ANTIMETABOLITE CHEMOTHERAPY

Gemcitabine

intravenous (IV) infusion



Interferes with deoxyribonucleic acid (the cell's genetic material) (DNA) synthesis

Gemcitabine is converted inside cells into forms that disrupt DNA building blocks and DNA copying. It can affect normal blood-cell production as well as cancer cells.

Activated inside cells → DNA production disrupted → Cell division slowed

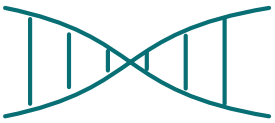
What to watch for

Report fever, unusual bruising, breathlessness, swelling or reduced urine urgently. Flu-like symptoms cannot safely be assumed to be a harmless drug effect.

PLATINUM CHEMOTHERAPY

Cisplatin

Cisplatin



Damages cancer-cell DNA

Cisplatin links strands of DNA, the instructions inside cells, making it harder for cancer cells to repair damage and divide. Healthy cells may also be affected. Kidney function, hearing, blood counts and nerve symptoms help the team decide whether the next dose is safe.

DNA damage → Cell repair disrupted → Cancer growth reduced

What to watch for

Report reduced urine, persistent vomiting, tinnitus, hearing change or new tingling.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding the evidence

The national regimen brings together research and specialist guidance for a specific treatment setting.

A randomised study of 480 patients compared this three-cycle induction approach followed by chemoradiotherapy with chemoradiotherapy alone. At three years, 85.3% versus 76.5% were alive without recurrence. Severe acute side effects were more frequent with induction (75.7% versus 55.7%). These group results do not predict your personal outcome.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment stages

Treatment stages

01 **Induction chemotherapy**
Day 1: both drugs; Day 8: gemcitabine. Every 21 days, up to 3 cycles.

02 **Next: chemoradiotherapy**
Separately prescribed

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

DAY 1: BOTH DRUGS; DAY 8: GEMCITABINE. EVERY 21 DAYS, UP TO 3 CYCLES.
Induction chemotherapy

Up to 3 cycles

Gemcitabine 1,000 mg/m² on Days 1 and 8, with cisplatin 80 mg/m² on Day 1, every 21 days for a maximum of 3 cycles.

SEPARATELY PRESCRIBED
Next: chemoradiotherapy

After induction and recovery review

The NCCP plan starts chemoradiotherapy 21–28 days after Day 1 of the last induction cycle, subject to review and recovery. This is measured from the first day, not from Day 8 or the end of that cycle. The later regimen has its own prescription.

The NCCP plan starts chemoradiotherapy 21–28 days after Day 1 of the last induction cycle, subject to review and recovery. This is measured from the first day, not from Day 8 or the end of that cycle. The later regimen has its own prescription.

BASELINE → TREATMENT → FOLLOW-UP

My tests and treatment checks

Routine safety tests, symptom-driven tests and response scans have different purposes.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00903 v1: retrieved official regimen

Tests in your plan

Full blood count

WHEN
Before each treatment.

Checks infection-fighting cells, oxygen-carrying cells and platelets for clotting.

Kidney and liver tests

WHEN

Day 1: full blood count, kidney and liver tests. Day 8: full blood count. Audiology is arranged when clinically indicated; extra tests depend on symptoms and results.

Checks treatment handling and organ safety.

Hearing assessment

If part of your plan

WHEN

When clinically indicated.

Assesses existing or new hearing changes and tinnitus.

THE WHOLE TREATMENT JOURNEY

Gemcitabine and cisplatin induction before chemoradiotherapy: treatment and checks

Induction chemotherapy before concurrent chemoradiotherapy for locally advanced nasopharyngeal cancer.

Gemcitabine 1,000 mg/m² on Days 1 and 8, with cisplatin 80 mg/m² on Day 1, every 21 days for a maximum of 3 cycles.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visits	Between visits	Next cycle / phase	After treatment
Blood and organ checks NCCP	Full blood count, kidney and liver tests; review medicines, allergies, weight, oral intake and symptoms. Review nerve symptoms and hearing; audiology when indicated. Cisplatin eligibility includes adequate kidney function.	Day 1: full blood count, kidney and liver tests. Day 8: full blood count. Audiology is arranged when clinically indicated; extra tests depend on symptoms and results.	Extra tests may be needed for symptoms or abnormal results.	Review results, recovery and earlier toxicity before another dose.	The team follows recovery, response and persistent side effects. Scans and tumour tests follow the individual plan; there is no universal scan or tumour-marker schedule in this regimen.

Care / check	Before starting	Treatment visits	Between visits	Next cycle / phase	After treatment
Treatment and appointments NCCP	Confirm the complete treatment plan.	Gemcitabine runs over 30 minutes, followed on Day 1 by cisplatin over 1 hour. Prescribed fluids before and after cisplatin lengthen the visit. Day 8 is gemcitabine alone.	Follow written medicine and contact instructions.	The NCCP plan starts chemoradiotherapy 21–28 days after Day 1 of the last induction cycle, subject to review and recovery. This is measured from the first day, not from Day 8 or the end of that cycle. The later regimen has its own prescription.	Record completion and the named follow-up team.
 **Mouth, weight and swallowing** NCCP	Assess symptoms, teeth, eating and drinking.	Review weight, intake and mouth symptoms.	Call for difficulty swallowing, worsening pain or weight loss.	Dietitian and swallowing support according to need.	Continue rehabilitation and dental care if advised.

What each check is for

1

Treatment course

Day 1: both drugs; Day 8: gemcitabine. Every 21 days, up to 3 cycles.

This chemotherapy is given before chemoradiotherapy to improve cancer control. Induction is the first stage of the planned pathway, and does not replace the later treatment.

Ask / act: Confirm active medicines, safety checks and support plan.

2



Review / next phase

After the planned course and recovery review

Review response and recovery.

Ask / act: Confirm actual dates and the next prescription.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before your first visit

Full blood count, kidney and liver tests; review medicines, allergies, weight, oral intake and symptoms. Review nerve symptoms and hearing; audiology when indicated. Cisplatin eligibility includes adequate kidney function.

STEP 2

Time in the unit

Gemcitabine runs over 30 minutes, followed on Day 1 by cisplatin over 1 hour. Prescribed fluids before and after cisplatin lengthen the visit. Day 8 is gemcitabine alone.

STEP 4

Leaving the unit

Confirm the next tests, medicines, treatment dates and verified daytime and out-of-hours contact numbers.

STEP 5

Practical help

Ask about transport, work, costs, communication needs and whether a support person can attend.

Your exact treatment schedule

Tests and preparation

During treatment

Day 1: full blood count, kidney and liver tests. Day 8: full blood count. Audiology is arranged when clinically indicated; extra tests depend on symptoms and results.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Gemcitabine	 **Day 1** Gemcitabine 1000 mg/m² on Day 1.
Cisplatin + fluids	 **Day 1** Cisplatin 80 mg/m² on Day 1, with prescribed hydration.
Gemcitabine	 **Day 8** Gemcitabine 1000 mg/m² on Day 8 after safety review.
Recovery and review	 **Days 9–21** Follow the support plan and report symptoms promptly.

Prescribed timing: Gemcitabine 1,000 mg/m² on Days 1 and 8, with cisplatin 80 mg/m² on Day 1, every 21 days for a maximum of 3 cycles.

Review and next step: The NCCP plan starts chemoradiotherapy 21–28 days after Day 1 of the last induction cycle, subject to review and recovery. This is measured from the first day, not from Day 8 or the end of that cycle. The later regimen has its own prescription.

Safety throughout: Symptoms and urgent-contact advice apply throughout treatment and after the last dose.

> **Why these bands stay broad:** This is a reference cycle, not a personal appointment calendar. Side effects can occur on any day; the recovery interval is not a guarantee of safety. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00903 v1: retrieved official regimen ↗

> **There are no guaranteed "safe days".** Use your treating service's written temperature threshold and urgent-contact plan. Fever, shivering or sudden illness can be serious even without a high temperature. Do not wait for the next visit. Severe chest pain, severe breathing difficulty, collapse or a seizure needs emergency help: call 112 or 999.

Between-treatment notes

Day 1: Day 1: treatment

Gemcitabine runs over 30 minutes, followed on Day 1 by cisplatin over 1 hour. Prescribed fluids before and after cisplatin lengthen the visit. Day 8 is gemcitabine alone. Report new symptoms before dosing.

Day 2–7: Between the first two visits

Follow prescribed support medicines. Blood counts and side effects may change between visits; this is not a guaranteed safe period. Call the team promptly if unwell. Keep the Day-8 blood-test and treatment appointment.

Day 8: Day 8: gemcitabine

Day 1: full blood count, kidney and liver tests. Day 8: full blood count. Audiology is arranged when clinically indicated; extra tests depend on symptoms and results. Day-8 decisions use their own blood-count and toxicity rules.

Day 9–21: Recovery and next review

Keep the next test and treatment dates. Fatigue and low blood counts may occur between visits. Do not wait for the next visit if unwell.

Support around the medicines

1 **Preventing and treating sickness**

Use the prescribed medicines for the active treatment day. The cisplatin combination visit has high sickness risk; Day 8 is gemcitabine alone and has a separate plan.

2 **Fluids with cisplatin**

Cisplatin needs prescribed fluids before and after treatment. Follow your individual drinking plan for the next 24 hours; heart or kidney conditions may change it. Call if vomiting prevents drinking or urine falls.

3 **Mouth care and nutrition**

Follow your unit’s oral-care plan. Tell the team early about mouth pain, weight loss, choking or difficulty eating and drinking. Dietetic and swallowing support can help.

4 **Medicine review**

Bring prescription, non-prescription and herbal medicines. Check interactions, allergies and any changes with the oncology pharmacist.

5 **Recovery and practical support**

Ask about fatigue, work, transport, caring responsibilities, emotional support and help with appointments.

Understanding your diagnosis

Tumour tests and safety tests

Your team explains cancer-specific tests, including Epstein–Barr virus tests where relevant in nasopharyngeal cancer. These are distinct from blood tests used for medicine safety. No universal tumour-marker schedule is prescribed here.

Your treatment aim and choices

Deciding together

Discuss expected benefit, possible lasting effects, suitable alternatives and the option of declining treatment. Tell the team what matters most to you.

Symptoms and when to call

Between visits

Keep a record of symptoms and medicines taken. Do not wait for a routine appointment when symptoms are worsening.

Mouth, eating and swallowing

Speak up early

Tell the team about pain, dry mouth, taste changes, choking, reduced intake or weight loss. Ask for a personalised oral-care, dietetic and swallowing plan.

Radiation-related care

If you are having head and neck radiation, follow the radiation team's dental, skin, saliva and swallowing advice during and after treatment. Radiation appointments and late-effect checks have their own plan.

Living with treatment

Activity and rest

Pace activity according to symptoms. Ask for help with fatigue, sleep, work, travel or caring responsibilities.

Sexual health and fertility

Discuss fertility preservation before treatment where appropriate. Ask about contraception, pregnancy, breastfeeding and medicine-specific timeframes.

Infusions and access

Your infusion arrangement

Line problems

Report pain, redness, swelling, leaking or a wet dressing. Do not adjust infusion equipment yourself; contact the unit.

After this course

Recovery and follow-up

The team follows recovery, response and persistent side effects. Scans and tumour tests follow the individual plan; there is no universal scan or tumour-marker schedule in this regimen.

Questions for your team

My plan

What is the treatment aim? Which tests are mandatory? What happens if a treatment day is missed?

My support

Which medicines do I take and when do I stop them? Whom do I contact overnight?

Questions worth asking

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

Try explaining the plan back: Can you explain your treatment days, required blood tests, support medicines and whom to call if unwell? Ask the team to write down any unclear part.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1: combination

Active medicines: Gemcitabine + Cisplatin

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg PO OD	Day 1 Clinician/pharmacy draft; reconcile to the individual prescription.
Ondansetron	16 mg PO OD	Day 1 Clinician/pharmacy draft; reconcile to the individual prescription.
Dexamethasone	12 mg PO OD	Day 1 Clinician/pharmacy draft; reconcile to the individual prescription.
Olanzapine	5 mg PO OD	Day 1 Clinician/pharmacy draft; reconcile to the individual prescription.
Aprepitant	80 mg PO OD	Days 2-3 Clinician/pharmacy draft; reconcile to the individual prescription.
Dexamethasone	8 mg PO OD	Days 2-4 Clinician/pharmacy draft; reconcile to the individual prescription.
Metoclopramide	10 mg PO TDS	Subsequent days / as directed Clinician/pharmacy draft; reconcile to the individual prescription.
Olanzapine	5 mg PO OD	Days 2-4 Clinician/pharmacy draft; reconcile to the individual prescription.

Additional phase-specific support

- Gemcitabine runs over 30 minutes, followed on Day 1 by cisplatin over 1 hour. Prescribed fluids before and after cisplatin lengthen the visit. Day 8 is gemcitabine alone.
- Day 1: full blood count, kidney and liver tests. Day 8: full blood count. Audiology is arranged when clinically indicated; extra tests depend on symptoms and results.
- The NCCP plan starts chemoradiotherapy 21–28 days after Day 1 of the last induction cycle, subject to review and recovery. This is measured from the first day, not from Day 8 or the end of that cycle. The later regimen has its own prescription.

- The local CUH source is retained as a reference list, not as the national antiemetic standard. Nystatin/chlorhexidine are displayed as local mouth-care adjuncts, not universal antiemetic or mucositis prophylaxis. Verify current local oral-care policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Day 8: gemcitabine only

Active medicines: Gemcitabine

Emetogenic risk: low

Medicine	Dose / route	When
Ondansetron	8 mg PO OD	Day 1 Clinician/pharmacy draft; reconcile to the individual prescription.
Metoclopramide	10 mg PO TDS	PRN Clinician/pharmacy draft; reconcile to the individual prescription.

Additional phase-specific support

- Gemcitabine runs over 30 minutes, followed on Day 1 by cisplatin over 1 hour. Prescribed fluids before and after cisplatin lengthen the visit. Day 8 is gemcitabine alone.
- Day 1: full blood count, kidney and liver tests. Day 8: full blood count. Audiology is arranged when clinically indicated; extra tests depend on symptoms and results.
- The NCCP plan starts chemoradiotherapy 21–28 days after Day 1 of the last induction cycle, subject to review and recovery. This is measured from the first day, not from Day 8 or the end of that cycle. The later regimen has its own prescription.
- The local CUH source is retained as a reference list, not as the national antiemetic standard. Nystatin/chlorhexidine are displayed as local mouth-care adjuncts, not universal antiemetic or mucositis prophylaxis. Verify current local oral-care policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Later chemoradiotherapy: separate prescription

Active medicines:

Emetogenic risk: Minimal

The later chemoradiotherapy regimen needs its own prescription and support plan; no induction medicines carry over.

Additional phase-specific support

- Gemcitabine runs over 30 minutes, followed on Day 1 by cisplatin over 1 hour. Prescribed fluids before and after cisplatin lengthen the visit. Day 8 is gemcitabine alone.
- Day 1: full blood count, kidney and liver tests. Day 8: full blood count. Audiology is arranged when clinically indicated; extra tests depend on symptoms and results.
- The NCCP plan starts chemoradiotherapy 21–28 days after Day 1 of the last induction cycle, subject to review and recovery. This is measured from the first day, not from Day 8 or the end of that cycle. The later regimen has its own prescription.
- Use current local supportive-care policy; review every medicine before prescribing.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

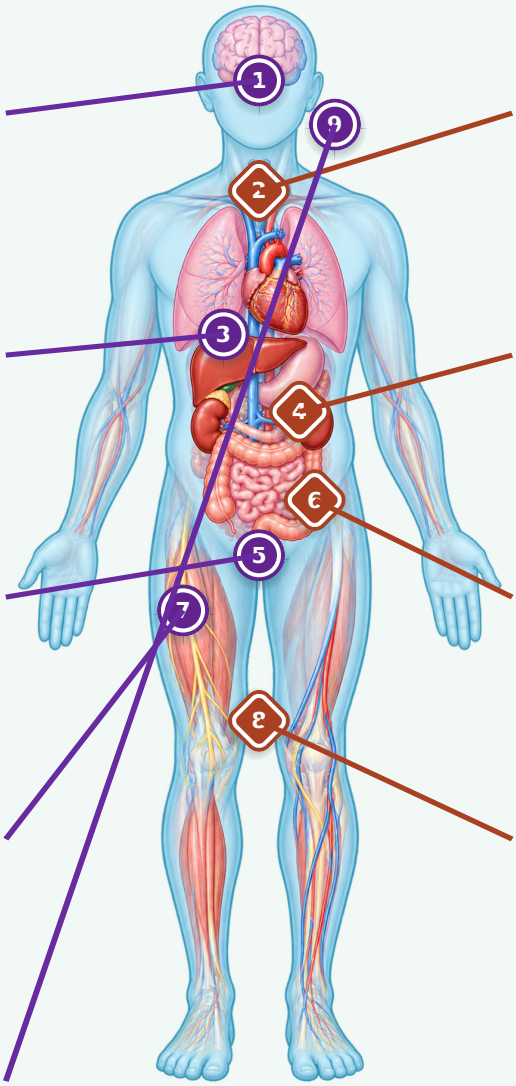
1 Fatigue and concentration
Gemcitabine + Cisplatin
Tiredness can affect activity and concentration. Balance activity and rest; report sudden confusion.

3 Breathing
Gemcitabine + Cisplatin
New or worsening breathlessness, cough or chest symptoms need urgent advice; lung inflammation can be serious.

5 Sickness and bowels
Gemcitabine + Cisplatin
Take prescribed anti-sickness medicines. Report persistent vomiting, diarrhoea, dehydration or inability to drink.

7 Skin and infusion reactions
Gemcitabine + Cisplatin
Report severe rash, blistering, facial swelling or breathing symptoms promptly. Tell staff about pain at the infusion site.

9 Hearing and nerves
Cisplatin
Report ringing, hearing loss, numbness or tingling before the next dose; some effects can persist.



2 Mouth and swallowing
Gemcitabine + Cisplatin
Report soreness, ulcers or difficulty swallowing. Ask early about mouth care, pain relief and nutrition.

4 Blood and infection
Gemcitabine + Cisplatin
Fever, shivering, sudden illness, unexplained bruising or bleeding needs urgent oncology advice.

6 Kidneys and fluid balance
Gemcitabine + Cisplatin
Report reduced urine, swelling or poor fluid intake. Blood tests help detect kidney injury.

8 Fertility and pregnancy
Gemcitabine + Cisplatin
Discuss fertility preservation, contraception, pregnancy and breastfeeding before treatment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivering, sudden illness, unusual bleeding, new breathlessness, persistent vomiting or inability to drink needs urgent oncology advice. Severe chest pain, severe breathing difficulty, collapse or a seizure is an emergency: call 112/999.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00903 v1 · pTreatment / supportive care**
   https://healthservice.hse.ie/documents/6859/903_V1_Gemcitabine_1g_CISplatin_80.pdf
2. **NCI: head and neck cancers**
   https://www.cancer.gov/types/head-and-neck/head-neck-fact-sheet
3. **NCI: oral complications of cancer therapies, patient version**
   https://www.cancer.gov/about-cancer/treatment/side-effects/mouth-throat/oral-complications-pdq
4. **NCI: nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
5. **HSE: cancer treatment side effects**
   https://www.hse.ie/conditions/cancer/if-you-have-side-effects/
6. **NCCP supportive-care antiemetics for NCIS**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
7. **Zhang et al., NEJM 2019: gemcitabine/cisplatin induction**
   https://pubmed.ncbi.nlm.nih.gov/31150573/